



Routine Immunization

A birth right of every child

Vaccination schedule after PCV introduction

Age (Completed)	Vaccination schedule after PCV introduction
At Birth	BCG, OPV (0 dose), Hep-B (birth dose)
1 ½ month (6 weeks)	OPV-1, Rota-1, fIPV-1 ➡ PCV-1*, Pentavalent-1
2 ½ month (10 weeks)	OPV-2, Rota-2*, Pentavalent-2
3 ½ month (14 weeks)	OPV-3, Rota-3*, fIPV-2 ➡ PCV-2*, Pentavalent-3
9 months	Vit A, MR-1, ➡ PCV-B*, JE-1
16-24 months	OPV (booster dose), MR-2, JE-2*, DPT (1 st booster dose)
5-6 years (up to 7 yrs. of age)	DPT (2 nd booster dose)
10 years	Td
16 years	Td

**JE / ROTA / PCV in selected states / districts*

After Birth

Hepatitis

*Within 24 hours in Rt Anterolateral aspect of mid-thigh, **Intramuscular (I.M.)**.*



Zero OPV

*Within 15 days from birth, **Oral***



BCG

As early as possible but can be given up to 1 years,

Intradermal

If no scar develops after BCG, it should not be repeated.

BCG & Measles can be given either together or with a gap of 28 days.



6 weeks vaccination



OPV

**1st Dose
(Oral)**



RVV

**1st Dose
(Oral)**



IPV

**1st Dose
(Intradermal)**



PCV

(Intramuscular)



Pentavalent

1st Dose

Pentavalent Vaccine

If the child is more than 1 year, PENTA 1st dose Should not be given, DPT should be given.

If 1st dose of PENTA is given before 1st year & child is brought for 2nd dose after 1 year, it can be given.

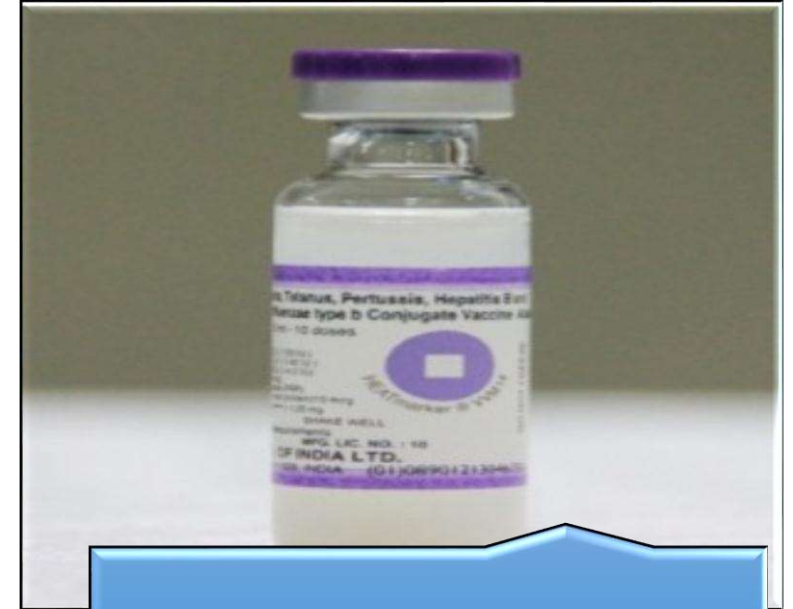
10 weeks vaccination



OPV 2nd Dose (Oral)



RVV 2nd Dose (Oral)



Pentavalent 2nd Dose

14 weeks vaccination



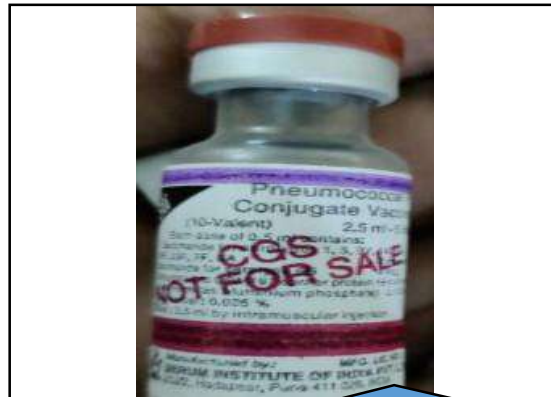
OPV 3rd Dose (Oral)



RVV 3rd Dose (Oral)



**IPV 2nd Dose
(Intradermal)**



**PCV 2nd Dose
(Intramuscular)**

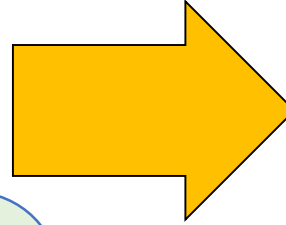


Pentavalent 3rd Dose

9 months to 24 months vaccination

9 months to 12 months

- Vitamin A 1st dose (1ml)
Oral
- MR / MMR 1st dose (in right arm, S.C.)
- **can be given until 5 years*
- PCV B (in right Thigh I.M.)
- JE 1st dose (in right thigh I.M.)
- **keep a gap of 2.5cm (1inch) between 2 injection sites*



16 months to 24 months

- Vitamin A 2nd dose (2ml)
- MR / MMR 2nd dose
- JE 2nd dose (in right thigh, I.M.) if applicable
- OPV booster & DPT 1st booster
- *If 3rd dose of DPT is given after 10 months, keep a gap of at least 6 months between 3rd dose & DPT 1st dose.*



5 years to 6 years

DPT 2nd booster should be given in left upper arm I.M.

Can be given until 7 years of age.

If planned to be given in school, inform parents earlier.



Age limits

Sr. No.	Vaccine Name	Doses	Age limits	Remarks
1	BCG		upto 1 year	BCG can be upto 1 year of age
2	IPV	1st dose	upto 1 year	IPV 1st dose can be given upto 1 year of age
3	RVV	1st dose	upto 1 year	RVV 1st dose should be given upto 1 year of age.
4	Pentavalent	1st dose	upto 1 year	1st dose of Pentavalent can be given upto 1 year of age
		2nd dose	before 7 years	2nd & 3rd dose of Pentavalent should be given as early as possible but before 7 years
		3rd dose		
5	PCV	1st dose	upto 1 year	1st dose of PCV can be given upto 1 year of age
6	OPV		upto 5 year	OPV can be given upto 5 years of age
7	MR / MMR		upto 5 year	MR / MMR can be given upto 5 years of age
8	DPT		upto 7 year	DPT can be given upto 7 years of age
9	JE	if applicable	upto 15 year	JE (if applicable) can be given upto 15 years of age.

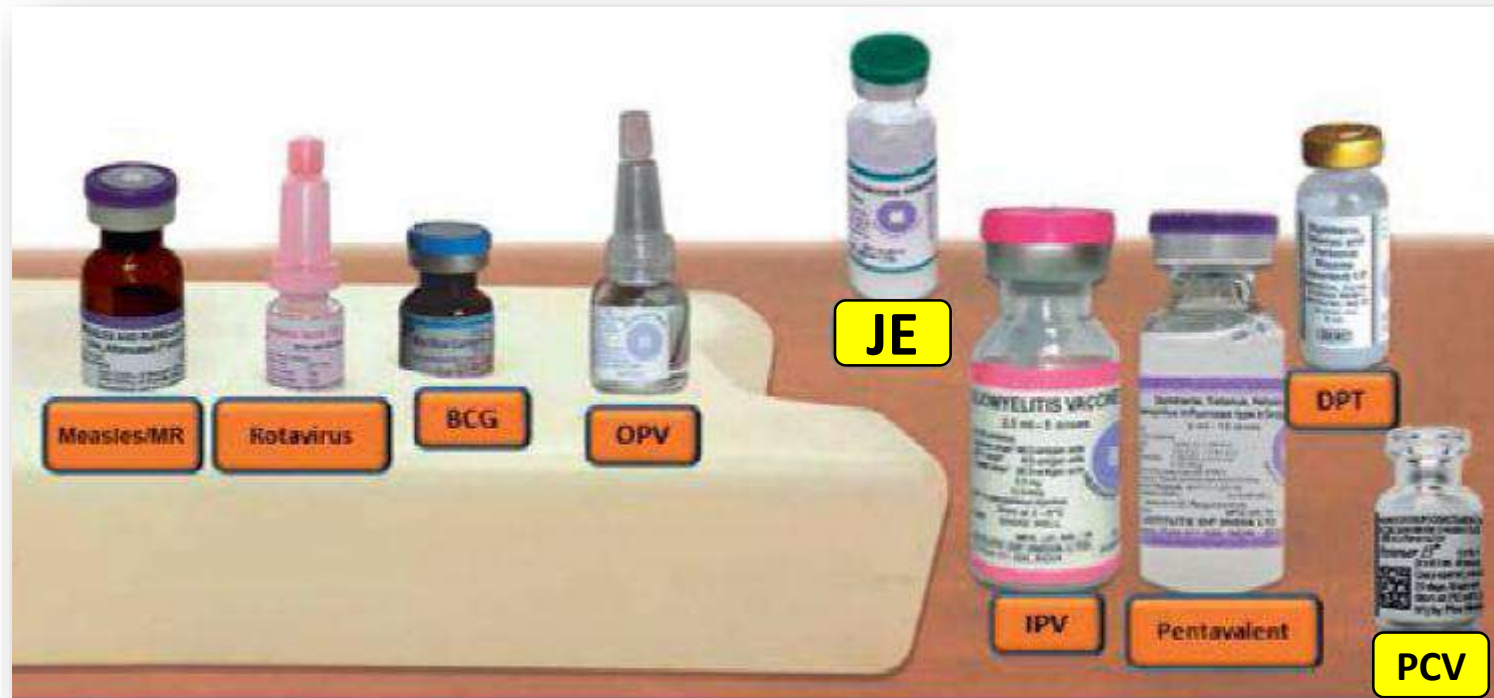
Which vaccines should be kept on an ice pack at the immunization site?

Newly Arrived Vaccine – PCV & JE (JENVAC) should never be kept on an ice pack.

On Ice Pack – BCG & Measles (*place them in the wells in ice pack*).

OPV and RVV should be placed on the surface of ice pack

Remember : IPV, HepB, TT, DPT, Pentavalent & PCV vaccines should never be kept on the ice pack.



Sites for vaccination in UIP



OPV : Oral Polio Vaccine;
Rota : Rotavirus Vaccine,
fIPV : fractional-dose IPV;
PCV : Pneumococcal Conjugate Vaccine;

Penta : Pentavalent vaccine;
Vit A : Vitamin A;
JE : Japanese Encephalitis vaccine;
MR vaccine : Measles-Rubella vaccine

4 Keys Messages



Which Vaccine has been given & what diseases it prevents ?



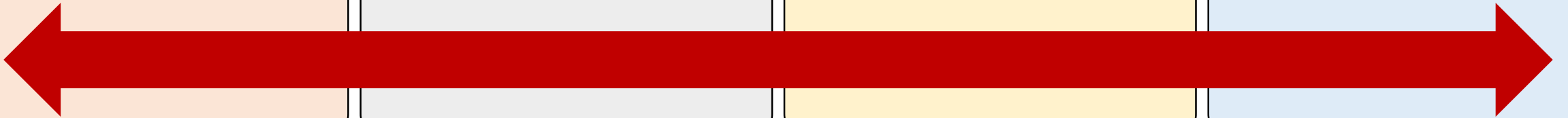
Minor side effects & what should be done?



When to come for the next dose?



To keep the Immunization card safe & to bring it during the next dose



Open Vial Policy

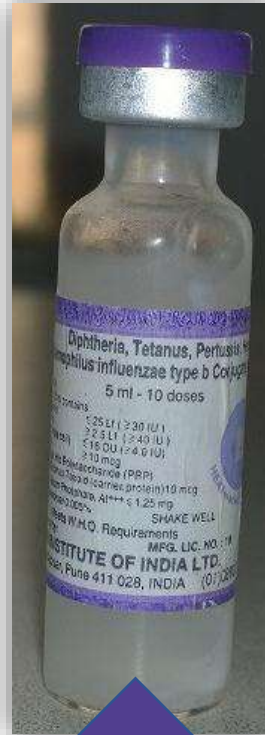


TT

OPV

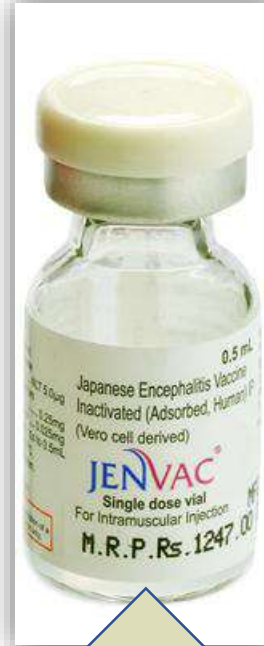
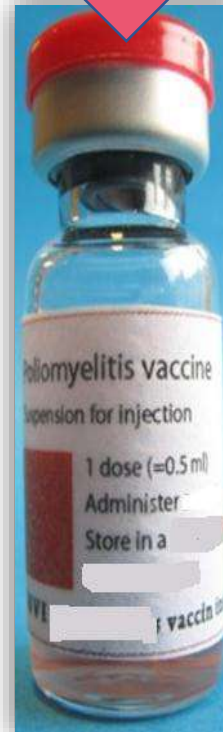
DPT

Hep
B



Penta

IPV



JE
(Jenvac)

PCV



Open Vial Policy is not Applicable to

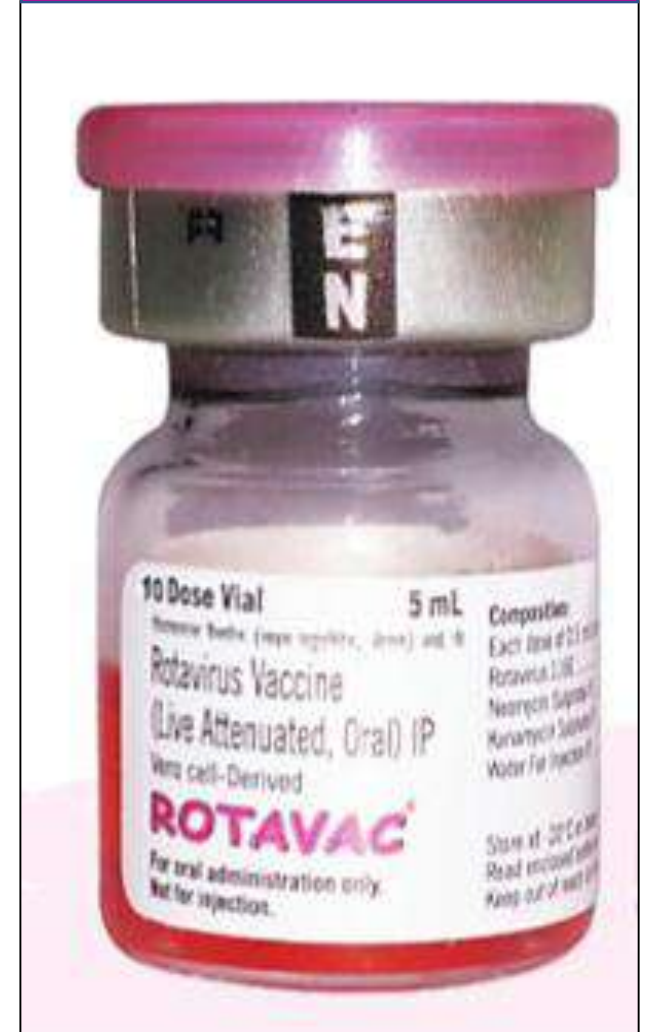
Measles- Rubella Vaccine



BCG Vaccine



Rotavirus Vaccine



Open Vial Policy

Mark date & time on ALL vaccine vials when opened for FIRST use



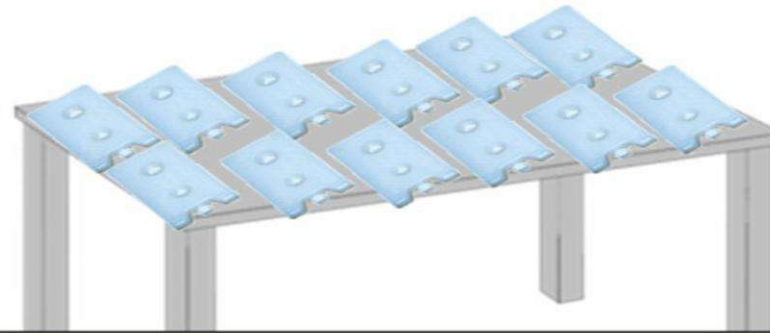
Why write time on all vials ?

- **Easy for ANM to remember**
- **Reconstituted vials (MR, RVV and BCG) to be discarded after 4 hours**
- **All other partially used vials to be brought back to vaccine storage point for use as per open vial policy.**

Conditioning of Ice Pack



When water ice packs are removed from the freezer, they may be at very low temperature between -15°C to -20°C



Place ice packs on the table and wait until the frost on the surface of ice packs melts



Place the ice packs first and then the vaccine



To see if ice packs are conditioned observe for sweating (water drops) on ice packs shake the ice packs to hear the water move inside



Sequence of Vaccination



Oral Polio Vaccine



RVV



IPV



MR / MMR



PCV



Pentavalent
or DPT is the
last



Good Session



**Time
8 am**

**List of
Beneficiaries**

**Pleasant
Atmosphere**

**Injection as
per
Guidelines**

**Sequences
of
Vaccination**

**4 Key
Messages**

**MCP
Card**

Record

BCG



BCG Vaccine given to a child with age >1 year

- *No need to worry but this is against guidelines*

A Child with weight $< 2\text{Kg}$

- *Give the vaccine*

The scar did not develop after vaccination

- *Vaccine should not be repeated*

The scar had developed but has now vanished

- *This is very rare but vaccine should not be given again*

Pentavalent Vaccine



Pentavalent Vaccine 1st dose given by mistake after 1 year of age.

- *No need to worry, continue with Penta – 2 & Penta – 3.*

Pentavalent Vaccine 1st dose given by mistake to a child < 6 weeks.

- *This should not be recorded 1st dose.*
- *All 3 doses should be given as per schedule.*
- *Giving Pentavalent before 6 weeks will result in reducing immunity against Hib diseases.*

Oral Polio Vaccine (OPV)



3 doses of Oral Polio Vaccine should be given along with 3 doses of Pentavalent vaccine

If they could not be given along with Pentavalent Vaccine, they can be given separately with a gap of at least 1 month between the doses.

OPV Booster should be given along with DPT Booster – 1

There is no 2nd Booster of OPV

- *No need to worry but this is against guidelines*

Pneumococcal Conjugate Vaccine (PCV)



A child < 1 years of age has come to the session site. Not received any vaccine earlier.

- *PCV – 1 should be given along with OPV – 1, RVV – 1, IPV – 1 & Penta – 1.*

A child < 1 year of age has come to the session site. Received OPV1, Penta1 earlier

- *The child should be given OPV2, RVV2 & Penta2*
- *This child will not receive PCV1*

A child > 1 year of age has come to the session site. Not received any vaccine earlier

- *The child should be given OPV1, MR / MMR vaccine 1 & DPT1*
- *This child will not receive PCV1*

MR / MMR Vaccine



A child > 1 year of age has come to the session site. Not received MR / MMR vaccine earlier

- *The child should be given MR / MMR vaccine 1st dose.*
- *The 2nd dose should be given 1 month after the 1st dose*

MR / MMR can be given until 5 years of age

It should be used only for 4 hours from reconstitution

Open Vial Policy is *not* applicable

MR / MMR Vaccine

If a child > 9 months old but < 1 year who has not received any vaccine earlier, give OPV 1, Vitamin A, MR / MMR vaccine, BCG, PCV1,& Pentavalent 1

If the mother refuses many injections, give OPV1, Vitamin A, PCV1 & MR / MMR on priority

Measles – Rubella (MR) vaccine is introduced in Maharashtra in 2018

A campaign was conducted before introducing MR vaccine in RI

9 months to 15 years children have been given 1 dose of MR vaccine in this campaign



Vitamin A

Enhances immune response when given along with MR / MMR vaccine

3rd Dose 6 months after 2nd Dose

4th to 9th Dose every 6 months until 5 years of age

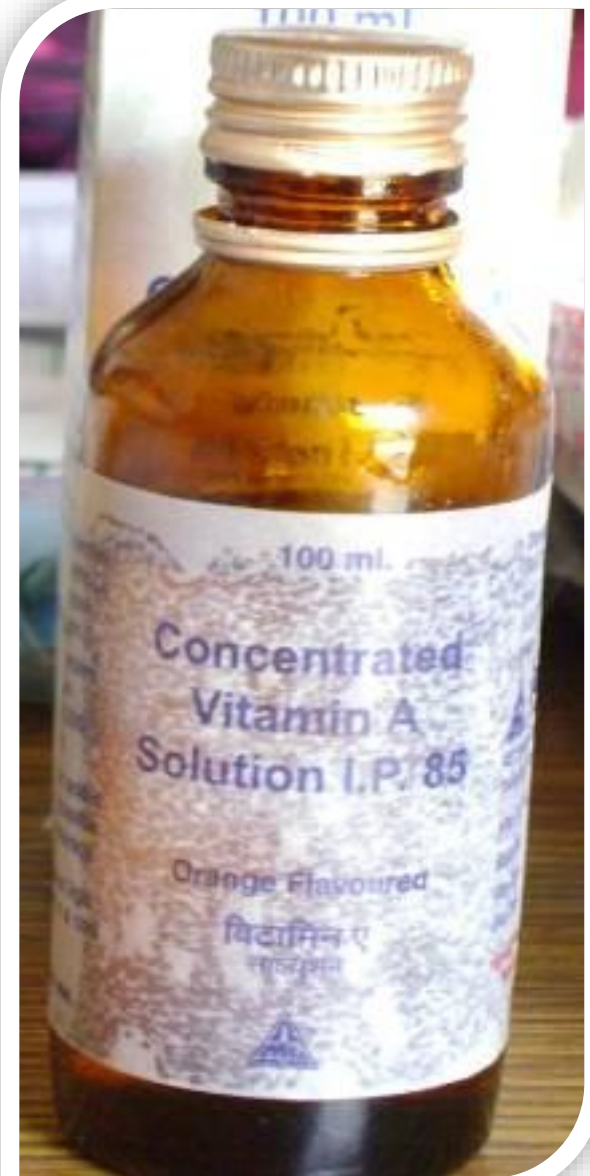
Spoon provided must be used.

In case capsules are provided *Blue (1ml)* below 12 months & *Red (2ml)* above 12 months

Over dosage must be avoided

In case capsules are provided Blue (1ml) below 12 months & Red (2ml) above 12 months

Over dosage must be avoided



JenVac (JE) Vaccine

A child > 1 year of age has come to the session site. Not received JE vaccine earlier

- *The child should be given JE vaccine 1st dose.*
- *The 2nd dose should be given 3 months after the 1st dose*

JE can be given until 15 years of age

JE vaccine is given in selected districts

New JE vaccine (JenVac) is given intramuscularly in anterolateral aspect of Right mid-thigh

Open Vial policy is allowed for JenVac

Liquid vaccine, No need of reconstitution

If 1st dose of old JE vaccine is given, 2nd dose of new JenVac can be given

Old JE



New JE



DPT Vaccine

To be given to a child > 1 year and who had not received any dose of Pentavalent vaccine

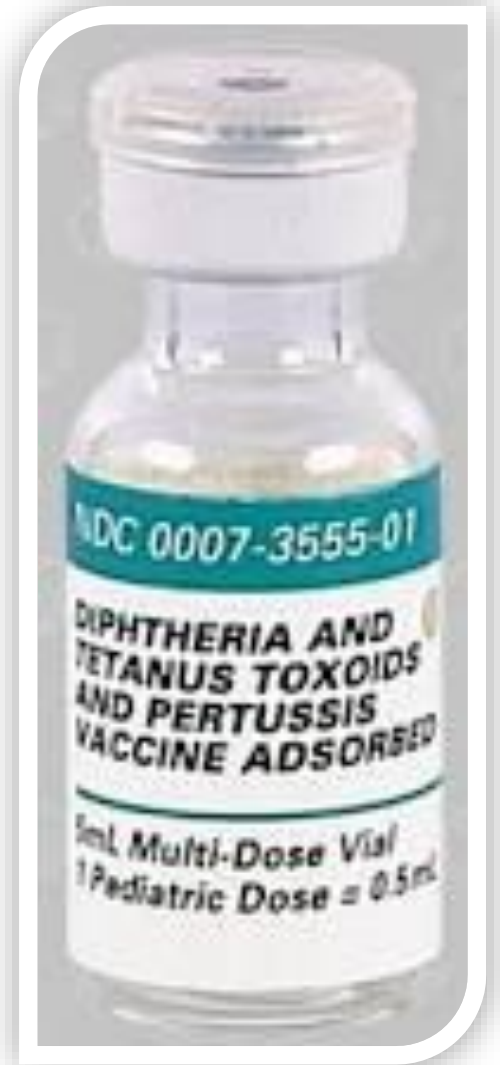
3 doses of DPT should be given with at least 1 month gap

DPT Booster 1 should be given at least 6 months after the 3rd dose.

If the child is > 16 months the dose of DPT given should not be counted as DPT Booster 1 unless the child had earlier received 3 doses of DPT

DPT Booster 2 should be given between 5 & 6 years

DPT vaccine can be given until 7 years of age



Inactivated Polio Vaccine

2 doses of IPV are given

1st dose with OPV 1 & 2nd dose with OPV 3 i.e. with a gap of 2 months

It can be given separately provided the child is < 1 year

There should be a gap of 2 months between 2 doses

0.1 ml given intra-dermally

1 vial will contain 25 or 50 doses

It should be given also in outreach sessions

Ignore the label

1st dose should not be given after 1 year of age





7mm

**Hold position
of needle and
depress plunger with
palm there will be a
feeling of resistance
if needle is placed
correctly
Watch bleb
appearing**



Tetanus diphtheria Vaccine (Td)

Pregnant Mothers

- 2 doses are given to pregnant mothers
 - *1 after registration of pregnant mother*
 - *2nd dose after 1 month*

If the mother becomes pregnant within 3 years of 2nd dose, only 1 dose is given called as Booster dose

Td 10 Years

Td 16 Years



GMSD / NATIONAL

STATE

DISTRICT

REGIONAL

COLD CHAIN POINT

SESSIION SITE



WIC



ILR



ILR



Vaccine Carrier

Till Cold chain point



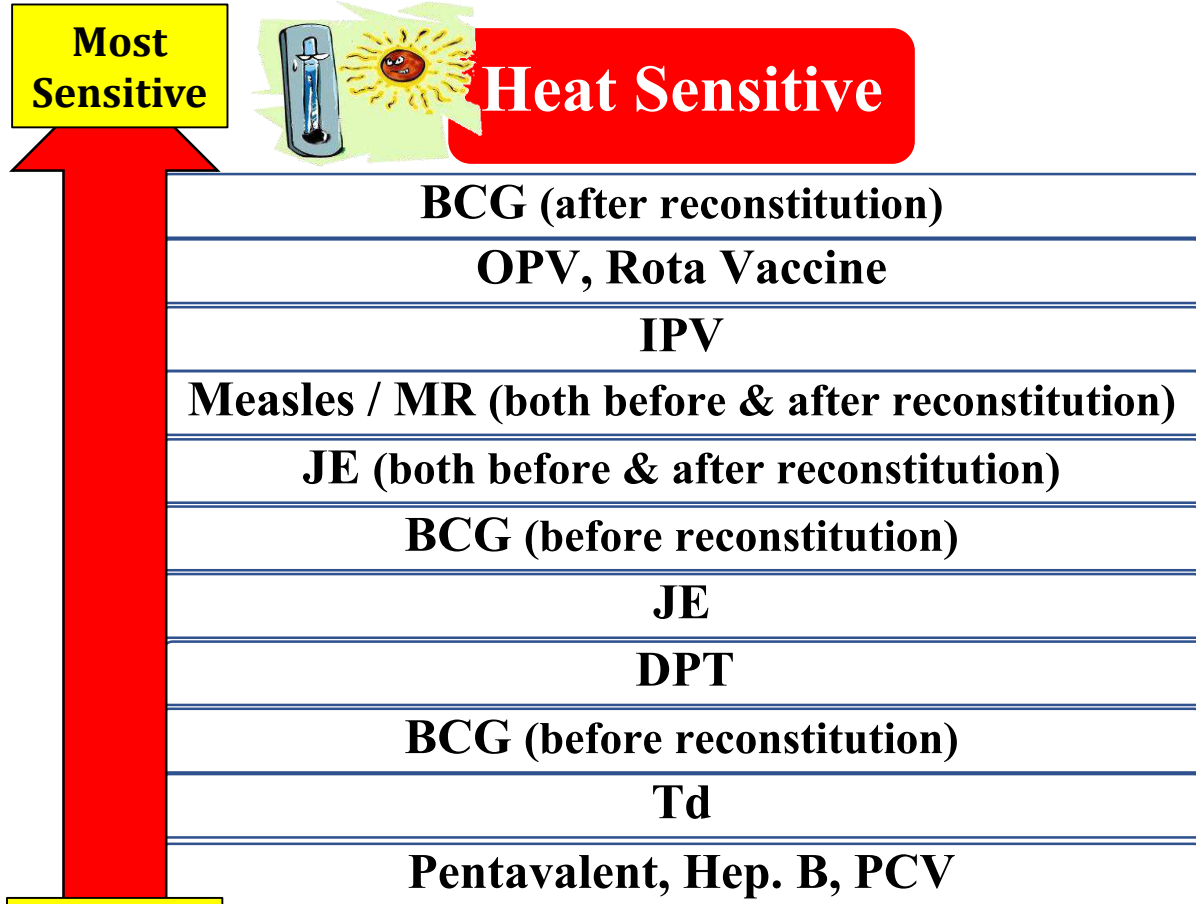
Vaccines should be transported in cold boxes with conditioned ice-packs .

Session Site

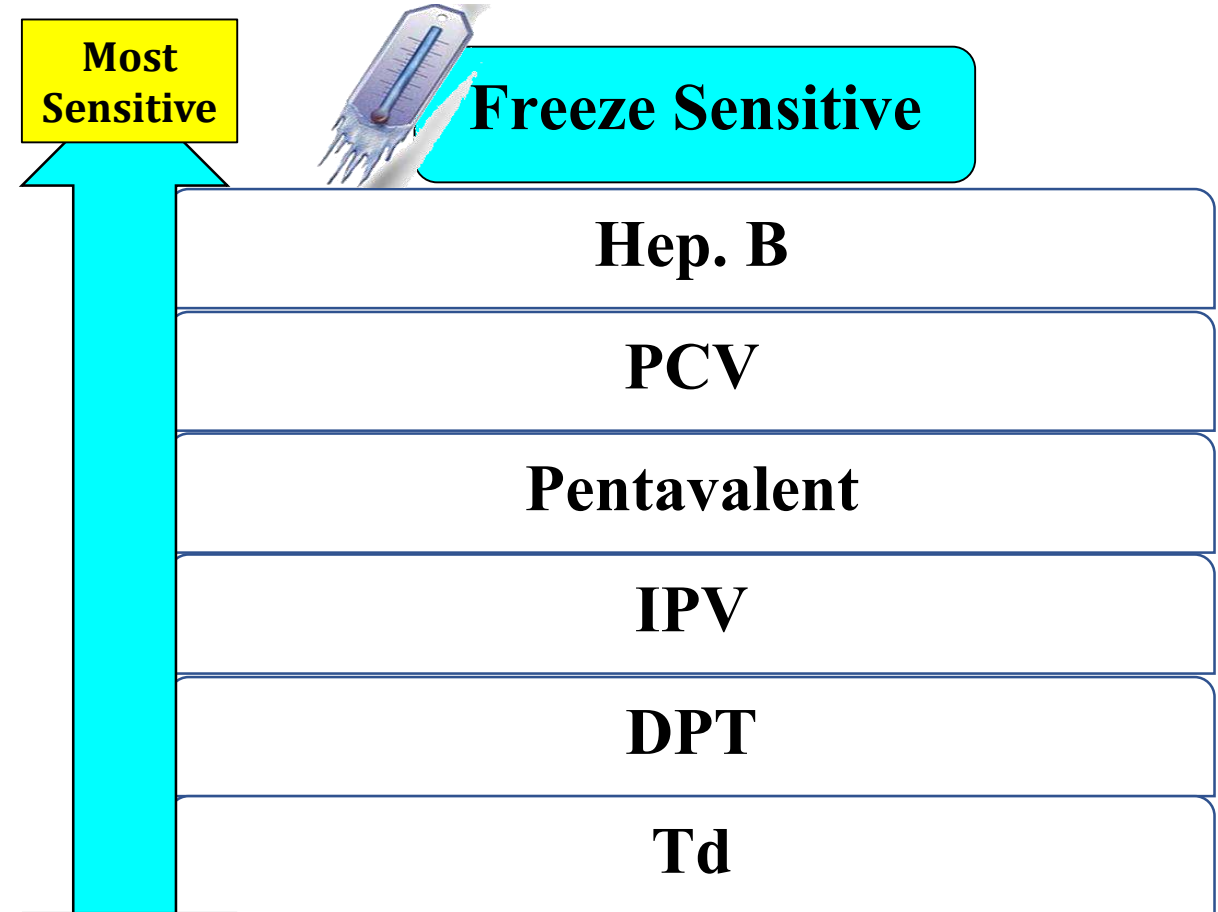


Vaccines should be transported to session sites in vaccine carrier with conditioned ice packs.

Sensitivity of vaccine



The damage caused by heat is cumulative and cannot be reversed by re-storing the vaccines under recommended temperature.

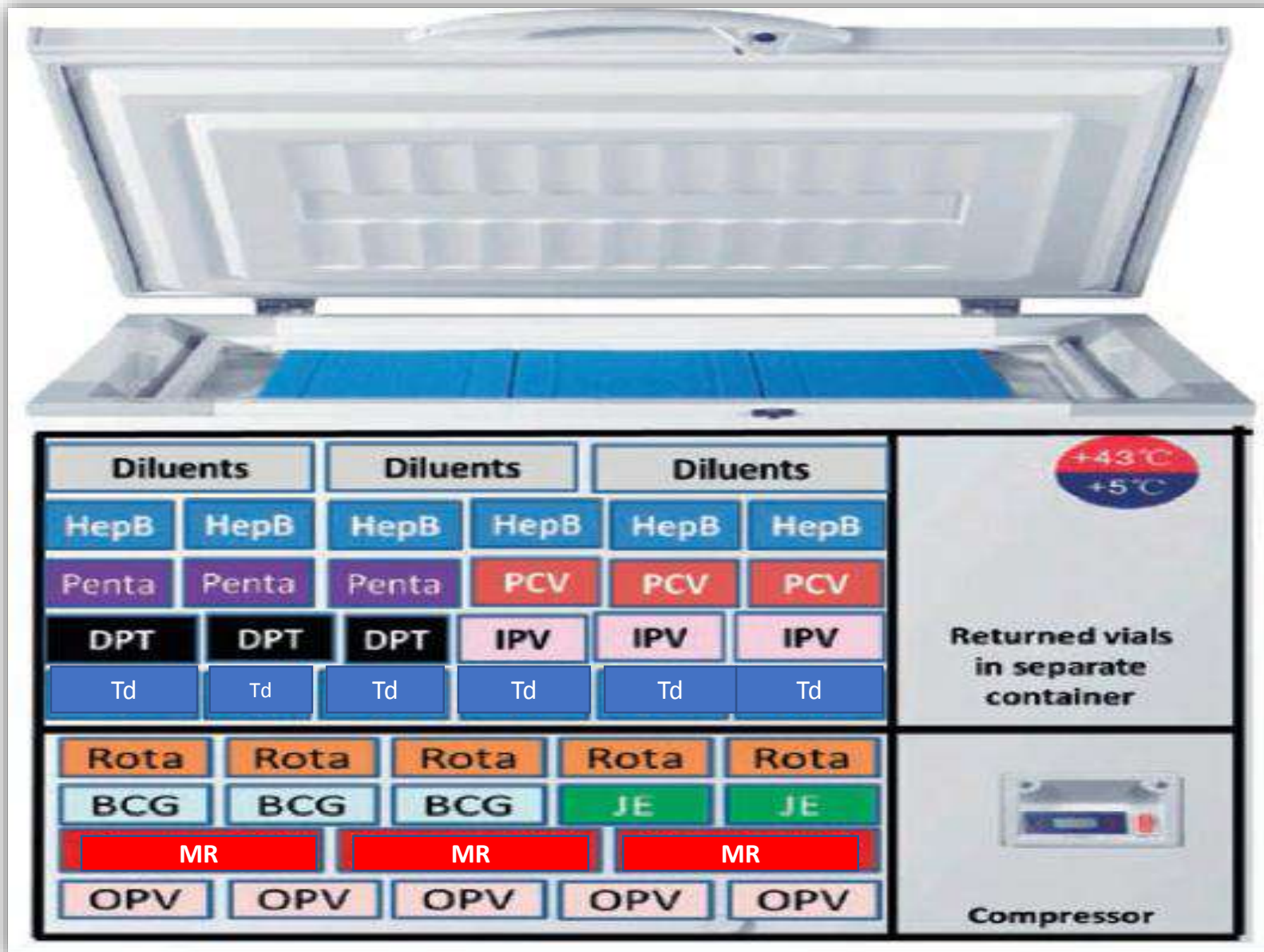


The effect of freezing is not cumulative, once frozen it is of no use.



Light Sensitive BCG, Measles/MR

Storing of vaccines in ILR



VVM

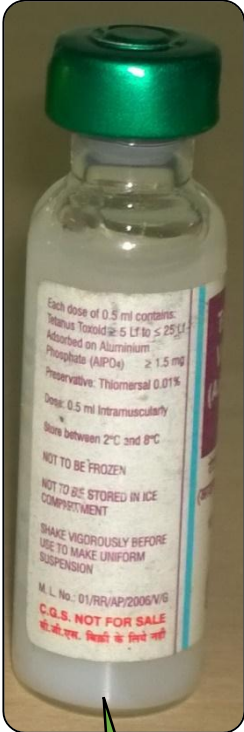
Ensure health workers are trained on appropriate handling of unpreserved multi-dose vials, as per the revised open vial policy guidelines by MoHFW

VVM: Marker to heat exposure to the vaccine

Start point		Square lighter than circle. If the expiry date has not passed. USE the vaccine.
End point		Square matches the circle. Even if not reached the expiry date, DO NOT use the vaccine.
End point exceeded		Square darker than the circle. Even if not reached the expiry date, DO NOT use the vaccine

Open Vial Policy

May be reused till 28 days of its opening



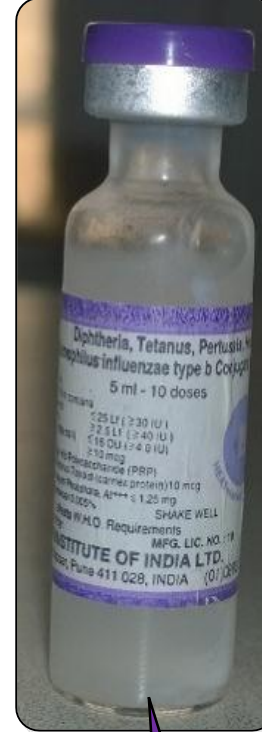
Td



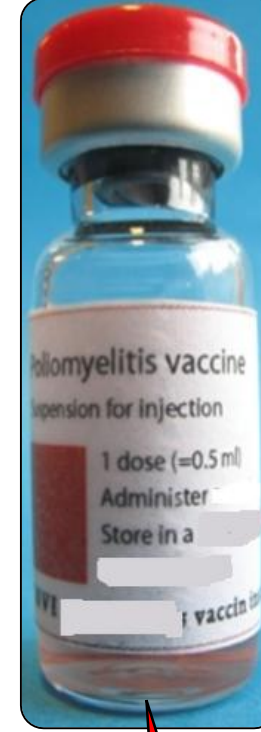
OPV

DPT

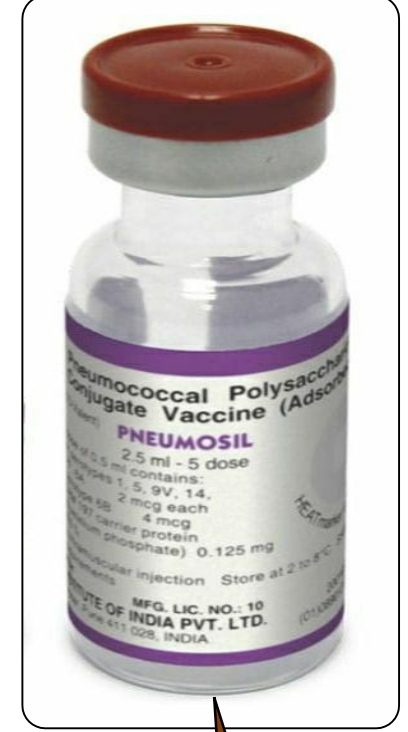
Heb-B



Penta



IPV



PCV

Open Vial Policy

Should be used only till 4 hours from the opening



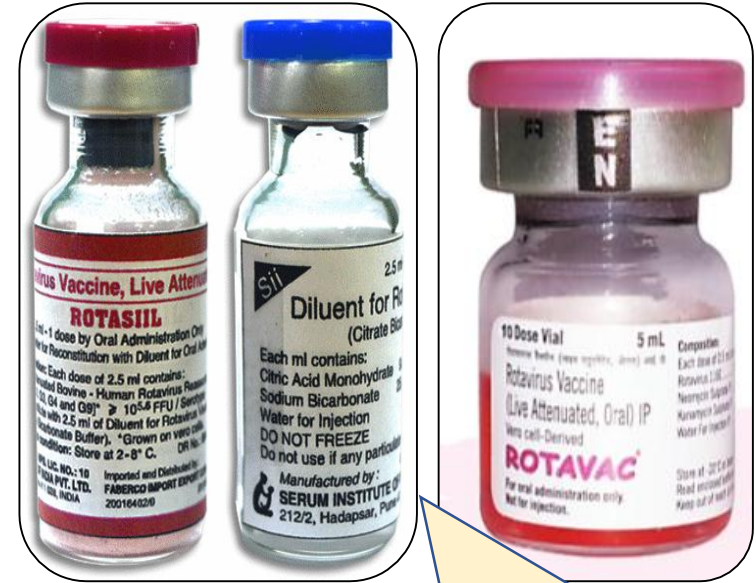
**Measles
Rubella**



BCG



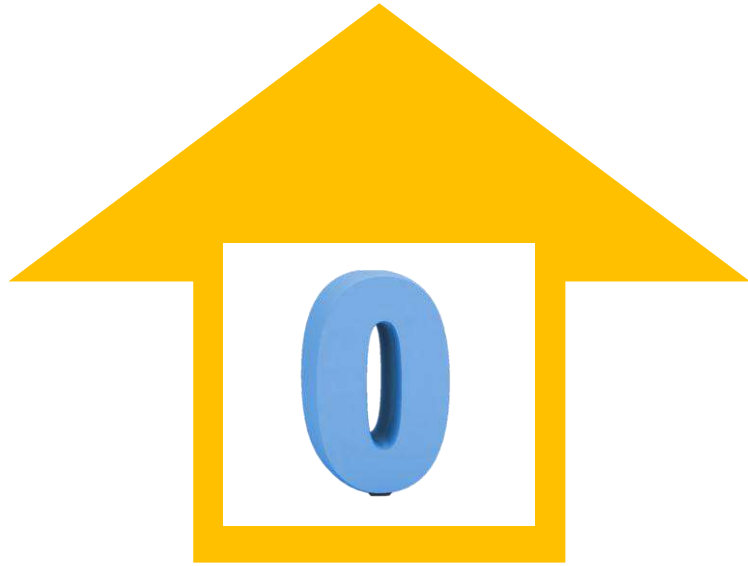
**Japanese
Encephalitis**



Rotavirus

Vaccine Management & Record Keeping

- ✓ Vaccine with the nearest expiry date are used first in a system known as EEFO (early expiry, first-out).
- ✓ **Expiry date should always be checked whenever a vial is opened.**
- ✓ Never use vaccines after the expiry date.
- ✓ **If you find frozen vaccine vial, do not use it and record it in the vaccine stock and distribution register.**
- ✓ If suspicion of freezing, do shake test.
- ✓ **Check VVM status. discard and document if not usable**
- ✓ Record the vaccine stock into stock register at the time of receiving of vaccine
- ✓ **Update vaccine distribution register at the time of issuing of vaccine to session**
- ✓ Enter vaccine stock into eVIN in real-time



Left Outs

- An eligible child who has not received any vaccine



Dropouts

- A child who was given a vaccine & then not followed further

Right Vaccine at Right Time

Preventing Dropouts

RI Micro-plan must be followed

Gentle Behavior

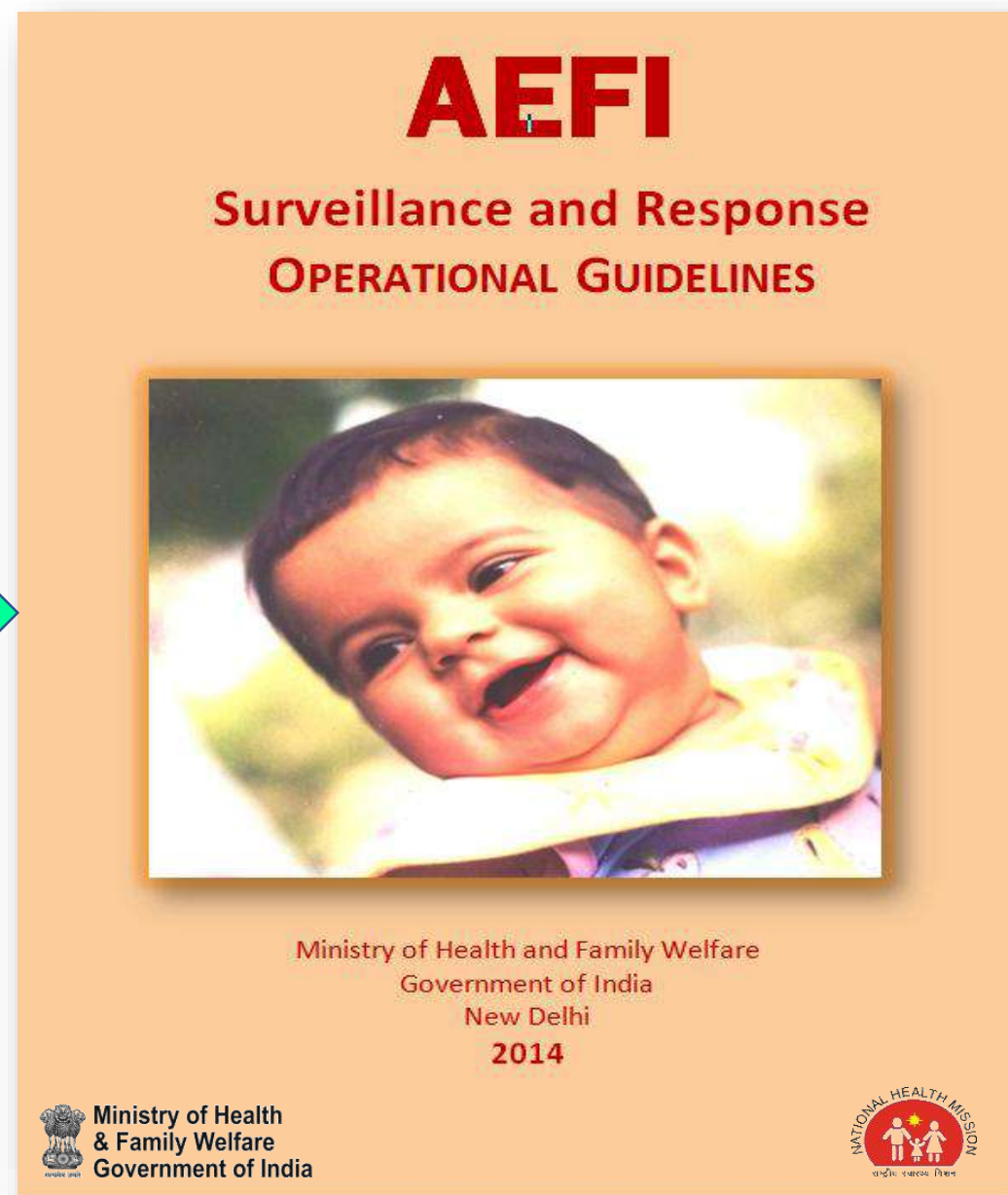
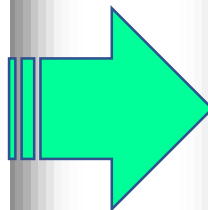
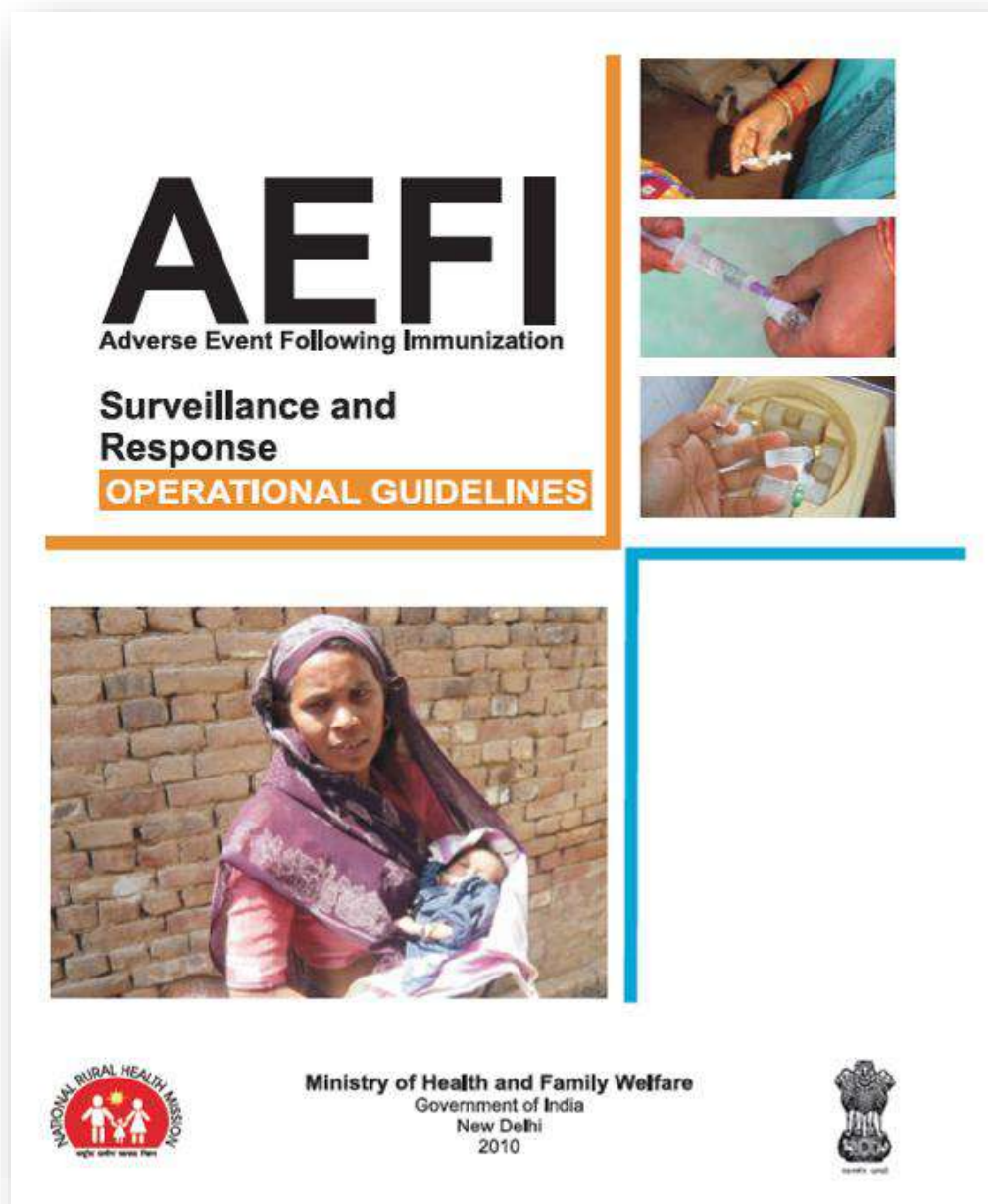
Explain about minor side effects

Tell the date of next vaccine

Explain the importance of vaccination



The revised Guidelines...



Minor reactions



Local

- Pain
- Swelling at injection site
- Redness at injection site



Systemic

- Fever < 102 F
- Irritability
- Malaise

Minor vaccine reactions are part of the body's immune system and occur more often than severe, but usually heal.

Serious and severe reactions

Serious Reactions

- Can be disabling and, rarely, life threatening
- Most do not lead to long-term problems
- Severe reactions include serious reactions but also include other severe reactions
- Must be reported
- Examples of severe reactions include
 - *Non- hospitalized recovered case of anaphylaxis*
- High grade fever (> 102 degree F),
 - *AFP*
- Hypotonic Hyporesponsive Episodes
 - *Sepsis*

Severe Reactions

- Results in death.
- Requires inpatient hospitalization.
- Results in persistent or significant disability.
- AEFI cluster

CASES TO BE REPORTED

Minor AEFI

Pain

Redness at site of injection

Swelling at site of injection

Fever < 102F

Irritability

Malaise

Serious AEFI

**Anaphylaxis but
hospitalization was not
required**

AFP

High grade Fever (>102F)

HHE

Sepsis

Severe AEFI

Death

Hospitalization

**Resulting in disability Cluster
(>1child)**

Annexure 1

AEFI CASE REPORTING FORM (CRF)																															
AEFI reporting ID: IND (AEFI) / __ST__ / DIS__ / YR__ / NUM__ (to be allotted by DIO)																															
Section A (To be submitted by MO within 24 hours of case notification to DIO)																															
State							District																								
Block/ward							Village/urban area																								
Name of reporting MO (person filling this form):														Today's date:																	
Posted at:							Designation:							Time of preparing this form: a.m./p.m.																	
Contact phone number: email:														Date case visited and examined/interviewed: __/__/__																	
Notified by (name):										Designation (please circle): health worker/government doctor/private practitioner/community/media/others (specify)																					
Date notified to MO: __/__/__																															
Patient's name																															
Date of birth DD/MM/YYYY							Age (in months): ____ months										Sex		Male		Female										
Mother's name																															
Father's name																															
Complete address of the case with landmarks (street name, house number, village, block, tehsil, pin no., telephone no.)																															
P i n - P h o n e -																															
Date of vaccination: __/__/__														Address of session site:																	
Time of vaccination: __: __ a.m./p.m.																															
Session: Routine (including SIW)*														Place of vaccination: govt health facility/outreach/private health facility/others ____																	
Campaign (SIA)-IPPI/MR/JE/others (specify): _____																															
Other _____																															
Names of vaccines received (write vaccine & diluent details in separate rows)				Dose no. (zero/first/s econd/etc. as applicable)		Name of manufacturer				Batch/lot No.				Expiry date		Date of opening of vial		Time of opening the vial (for reconstit uted vaccine)		No. of OTHER beneficiaries who received vaccine from the SAME vial in this session											
Date of first symptom				a		a		m		m		r		r		r		r		Time of first symptom		H		H		M		M		(a.m. p.m.)	
Hospitalization: No/yes — (Date)				a		a		m		m		r		r		r		r		Time of hospitalization		H		H		M		M		(a.m. p.m.)	
Name and address of hospital (if hospitalized):																															

*Special immunization week

Case Reporting Form (CRF) *2 pages*

Immunization Field Monitors (IFM)

Training conducted at Pune on 11th Feb & 12th Feb

They will be reporting to DRCHO & RMO (OR)

SMO will help in Technical Guidance

Weekly summary to be entered in excel sheet & should be sent to SMO office

Monthly report to be sent every month

Monitoring data to be entered in a software daily



IFM - Responsibilities

RI Monitoring – 5 in a week

RI Monitoring – Sessions & House to House

Facilitating RI Micro-planning

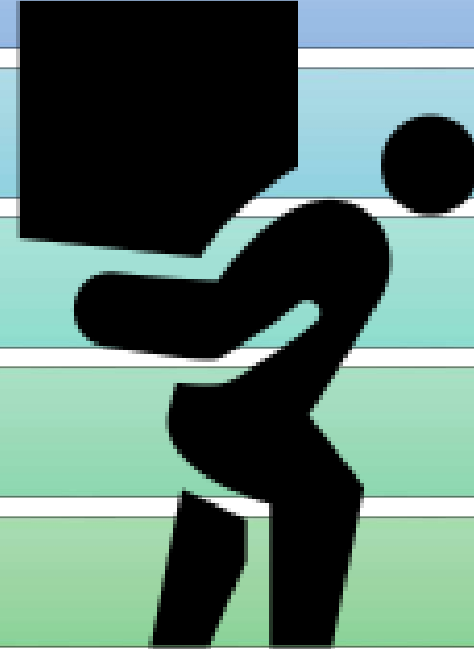
Validation of HRA

Cold Chain Monitoring

Facilitating Micro-planning in campaigns like Pulse Polio, MR campaign, IMI, Covid Vaccination Campaign etc.

Helping AFP & Measles surveillance

- **Visiting Reporting sites**
- **Tracking Stool collection**



Weekly plan & Monthly report

Annex: II

WEEKLY WORK PLAN FOR IMMUNIZATION FIELD MONITORS (IFMs)

Name: Umesh Wamanrao Tathe District: Akola

Blocks/Urban areas assigned: Akot, Akola, Balapur, Barshitakli

Week No.: 26 (27/06/2021 to 03/07/2021)

Day	Date	Area to be covered	Specific Task Assigned	Progress / Status	Follow up Plan	Appraisal / Remarks
Sunday	27/06/2021	Holiday	Holiday	Holiday	Holiday	Holiday
Monday	28/06/2021	Akot Block	Covid Vaccination Session & House to House	Done		
Tuesday	29/06/2021	Balapur Block	Covid Vaccination Session & House to House	Done		
Wednesday	30/06/2021	Barshitakli Block	Covid Vaccination Session & House to House	Done		
Thursday	01/07/2021	Akola Block	Covid Vaccination Session & House to House	Done		
Friday	02/07/2021	Akola Block	Covid Vaccination Session & House to House	Done		
Saturday	03/07/2021	Holiday	Holiday	Done	Holiday	Holiday

Signature of IFM:



Date of submission: 26/06/2021

- A plan will be submitted by IFM every week to **DRCHO THO SMO**
- Daily activities will be noted by IFM in their diaries
- A monthly report will be signed by THO & will be submitted to DRCHO & a copy to SMO
- Remuneration will be paid after DRCHO approves

Annexure III:

Monthly Performance Report of IFM for the month of May-2021

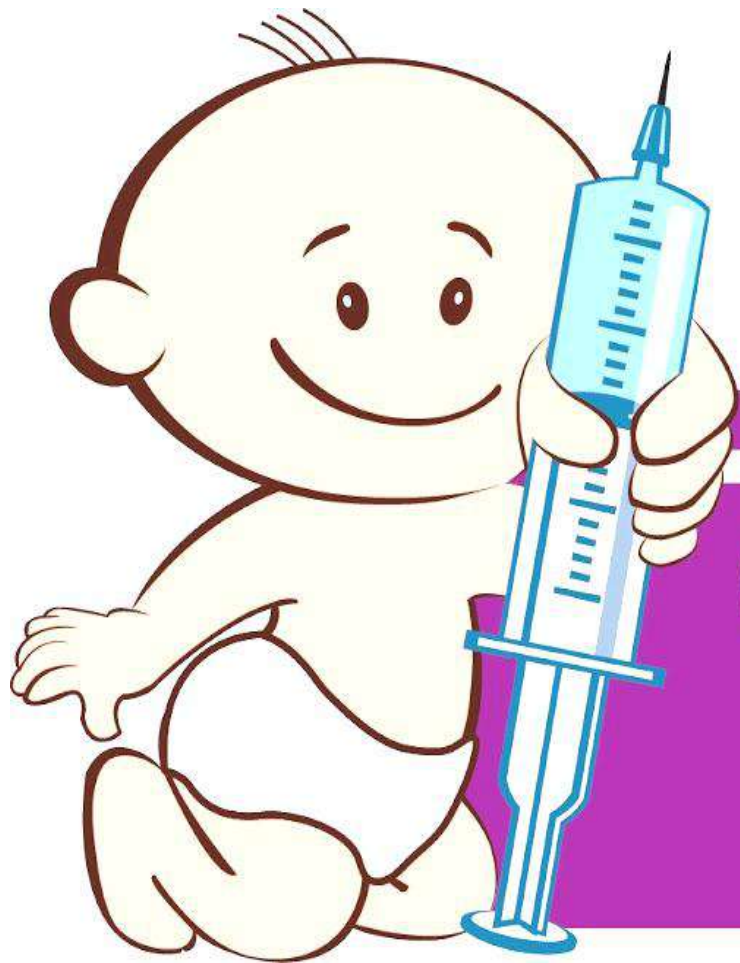
District: Akola Name of IFM: Umesh Wamanrao Tathe

Blocks/Urban areas assigned: Akot, Akola, Balapur, Barshitakli No of days Worked ---31 Days

Area of Work	Specific Activities	Achievement Numbers	Comments
A. Routine Immunization Strengthening	1a. Number of PHC/SDH/RH/HP visited in a month for RI micro-plan revision 1b. Number of HRAs Validated by IFM in a month 2a. Number of sessions monitored, 2b. Number of HTH area monitored, 2c. Number of PHCSDH/RH/HP monitored in a month for supportive supervision of cold chain 2d. Number of PHCSDH/RH/HP monitored in a month for communication planning monitoring	21 0 67 5 21 21	Covid Session Microplan RI Session - 8 Covid -57
B. Supplementary Immunization Activities and other vaccination campaigns like Mission Indradhanush and MR campaigns, etc	3. Number of PHCs/ SDH /RH /HP visited for micro-plan review and submission 4. Number of training / workshop / meetings supported and participated prior to each Pulse Polio round 5. Number of booths, Transit sites, HRAs and HH monitored during each round	0 0 0	
C. Acute Flaccid Paralysis and Measles surveillance	6. Number of reporting sites visited in a month 7. Number AFP cases for which stool collection tracked 8. Number of preliminary searches completed in flagged measles outbreaks in a month 9. Number of outbreak investigation supported and participated in a quarter	8 0 0 0	



Signature of IFM: Umesh Wamanrao Tathe



भूल न जाना,

**टीकाकरण
ज़रूर करवाना!**

धन्यवाद